

Patient ID: RSanchezPeriod: September 17 to November 20, 2024Report Date: November 21, 2024Coverage: Bed: 1163/1544h (75.3%)Chair: 1117/1544h (72.3%)

Patient's monitoring period was less than 90 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm
Very High HR > 110 bpm

Low HR < 45 bpm
Elevated Breathing > 24 brpm

Elevated HR > 95 bpm
High Breathing > 30 brpm

High HR > 100 bpm
Very High Breathing > 40 brpm

Patient clinical status over 65 days from Sep 17 to Nov 20

Elevated Breathing
Sep 25 to Sep 29 (5d)

Elevated Breathing
Oct 06 to Oct 11 (6d)

Elevated Breathing
Nov 05 to Nov 07 (3d)

Elevated Breathing
Nov 13 to Nov 20 (8d)

High Breathing
Sep 24 to Oct 03 (10d)

High Breathing
Oct 08 to Oct 09 (2d)

High Breathing
Oct 29 to Nov 06 (9d)

High Breathing
Nov 16 to Nov 20 (5d)

HR Breathing

Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: YELLOW

Vital	Avg	Min	Max
Heart Rate	57 bpm	49 bpm	71 bpm
Breathing	19 brpm	9 brpm	56 brpm

Limited data in the final 24h (15/24h); values reflect available readings.

Last 24 hours: two high breathing episodes in the afternoon; two elevated breathing episodes in the afternoon.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Nov 20	4 PM to 5 PM	4h	4	Elevated Breathing	26	17	45	Check SpO2 and lung sounds; assess for fluid overload or early infection
Nov 20	2 PM to 3 PM	2h	2	High Breathing (brief)	30	14	56	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

86 episodic events Detected, spanning 4 days total. Conditions: Elevated Breathing, Elevated Heart Rate, High Breathing, High Heart Rate, Low Heart Rate, Very High Breathing. Priority grade: Medium

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
86	100h	High Breathing	1	100%

Major Findings: Sustained high breathing (avg 35, peak 58)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Sep 24	1 PM to 2 PM	15h	12	High Breathing	35	12	58	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Oct 08	8 AM to 9 AM	3h	3	High Breathing	35	12	54	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Oct 29	9 AM to 10 AM	20h	16	High Breathing	35	16	46	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Nov 16	12 AM to 1 AM	7h	7	High Breathing	34	14	56	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Sep 25	7 AM to 8 AM	6h	6	Elevated Breathing	26	14	57	Check SpO2 and lung sounds; assess for fluid overload or early infection
Oct 06	1 PM to 2 PM	12h	10	Elevated Breathing	26	13	46	Check SpO2 and lung sounds; assess for fluid overload or early infection

+ 2 additional condition(s) across Elevated Breathing; details in monitoring data.

The P5 to P95 heart rate spread was 24 bpm (52 to 76), indicating significant cardiac variability.

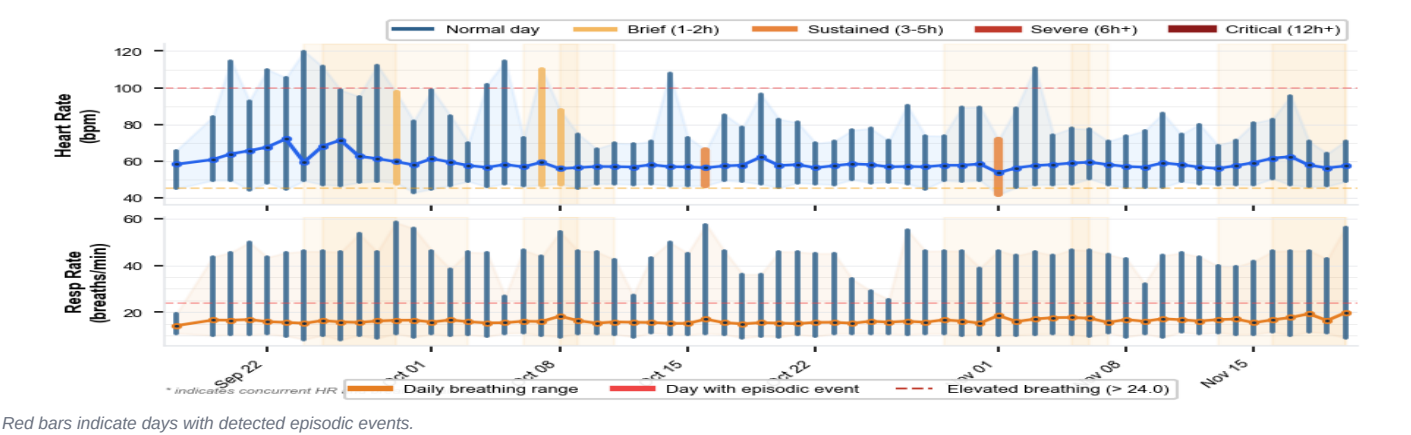
Suggested Clinical Review Actions

- High Breathing (avg 35 brpm, peak 46 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Elevated Breathing (avg 26 brpm, peak 46 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection

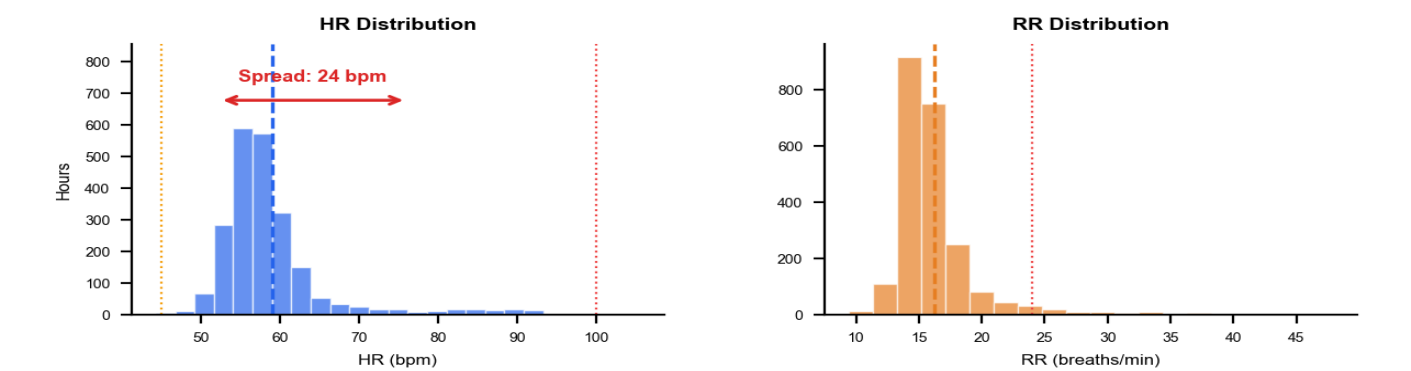
- High Heart Rate (avg 103 bpm, peak 115 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Elevated Heart Rate (avg 98 bpm, peak 120 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Very High Breathing (avg 44 brpm, peak 54 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

Clinical Pattern Observations:

- 1. **High variability:** HR spread 24 bpm (52 to 76).
- 2. **Clustered pattern:** Episodic events on 5 of 65 days (7%).
- 3. **Daytime pattern:** Episodes concentrated during waking hours.



Vital Sign Distribution



Monitoring Activity

